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Ankit Prakash

25 Year(s) Male

Reported on

September 06, 2026

Index

Ultra Full Body Checkup - Male

- Complete Blood Count (CBC) with ESR
- Hepatitis B Surface Antigen (HbsAg)
- Lactate Dehydrogenase (LDH)
- Immunoglobulin E (IgE)
- C-Reactive Protein (CRP)
- Ferritin
- Iron Profile
- Vitamin B12
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- Folic Acid | Folate | Vitamin B9
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Ultra Full Body Checkup - Male

- Microalbumin Creatinine Ratio
- Urine Complete Analysis
- Thyroid Profile, Free
- Triiodothyronine (T3) - Total
- Thyroxine (T4) - Total
- Thyroid Peroxidase Antibody (TPO)
- Cortisol, Random
- Testosterone
- Testosterone, Free
- Dehydroepiandrosterone Sulphate (DHEAS)
- Sex Hormone Binding Globulin (SHBG)
- Rheumatoid Factor (RF)
- Prostate Specific Antigen (PSA)
- Carcinoembryonic Antigen (CEA)
- Carbohydrate Antigen 19-9 (CA 19-9)
- Alpha Feto Protein (AFP)

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 01:11 PM



Test

Result

Biological Reference Interval

Blood Profile

Complete Blood Count (CBC) with ESR | Whole Blood EDTA

Red Blood Cells (RBC) Count

DC Impedance Method

6.10

4.5 - 5.5

mill/mm³

Hemoglobin (Hb)

Cyanide-free SLS method

16.2

13 - 17

g/dL

Hematocrit (HCT) | Packed Cell Volume (PCV)

Calculated

49.8

40 - 50

%

Mean Corpuscular Volume (MCV)

Calculated

81.6

83 - 101

fL

Mean Corpuscular Hemoglobin (MCH)

Calculated

26.6

27 - 32

pg

Mean Corpuscular Hemoglobin Concentration (MCHC)

Calculated

32.5

31.5 - 34.5

g/dL

Red Cell Distribution Width (RDW) CV

Calculated

14.5

11.6 - 14

%

Mentzer Index

Calculated

13.4

Beta Thalassemia trait: < 14
Iron deficiency anaemia: >= 14
Index Value

Sehgal Index

Calculated

1091.6

Beta Thalassemia trait: < 972
Iron deficiency anaemia: >= 972
Index Value

Total White Blood Cell Count (TC)

Flow Cytometry

10390

4000 - 10000

cells/mm³

Differential Count

Ankit Prakash

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Test	Result	Biological Reference Interval
Neutrophils Flow Cytometry	70.9	40 - 80 %
Lymphocytes Flow Cytometry	22.1	20 - 40 %
Monocytes Flow Cytometry	4.3	2 - 10 %
Eosinophils Flow Cytometry	2.5	1 - 6 %
Basophils Flow Cytometry	0.2	0 - 2 %
Absolute Neutrophil Count (ANC) Calculated	7367	2000 - 7000 /mm ³
Absolute Lymphocyte Count (ALC) Calculated	2296	1000 - 3000 /mm ³
Absolute Monocyte Count (AMC) Calculated	447	200 - 1000 /mm ³
Absolute Eosinophil Count (AEC) Calculated	260	20 - 500 /mm ³
Absolute Basophil Count (ABC) Calculated	21	0 - 100 /mm ³
Neutrophil Lymphocyte Ratio (NLR) Calculated	3.0	1 - 3 Ratio
Platelet Count DC Impedance Method	203	150 - 450 10 ³ /μL
Platelet Hematocrit Calculated	0.279	0.2 - 0.5 %
Mean Platelet Volume (MPV) Calculated	13.7	7 - 13 fL

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Test	Result	Biological Reference Interval
Erythrocyte Sedimentation Rate (ESR) Quantitative Capillary Photometry	4	0 - 10 mm/h

Clinical Significance - Complete Blood Count (CBC) with ESR

- Reference Ranges are in accordance with Dacie & Lewis Practical Hematology International Edition (12th)
- As per International Council for Standardization in Hematology's recommendations Differential Leucocyte counts are additionally reported in Absolute numbers in each cell per unit volume of blood.

Approved By

Dr Shachi
MBBS, MD (Pathology)
Pathologist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 07:43 PM



Test

Result

Biological Reference Interval

Inflammation and Infection Profile**Hepatitis B Surface Antigen (HbsAg)**

Serum, Chemiluminescent Microparticle Immunoassay (CMIA)

0.38

Nonreactive: < 0.80

Greyzone: 0.80 - 0.99

Reactive: >= 1.00

S/C Units

Lactate Dehydrogenase (LDH)

Serum, Twopoint-Rate-Pyruvate to Lactate

235

120 - 246

U/L

Immunoglobulin E (IgE)

Serum, Latex-Enhanced Immunoturbidimetry

77

0 - 100

IU/mL

C-Reactive Protein (CRP)

Serum, Non-Competitive Immunorate Assay

<5.0

<10

mg/L

Approved By

Dr Shivaprakash

MSc PhD (Medical Biochemistry)

Biochemist

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 02:55 PM



Test

Result

Biological Reference Interval

Nutritional Status

Ferritin

162.70

21.81 - 274.66

Serum, Chemiluminescent Microparticle Immunoassay (CMIA)

ng/mL

Iron Profile

Serum

Iron

80

49 - 181

Colorimetric - Pyridyl Azo

µg/dL

Total Iron Binding Capacity (TIBC)

405

261 - 462

Immunoturbidimetry Chromazurol-B

µg/dL

Unbound Iron Binding Capacity (UIBC)

325

110 - 370

Calculated

µg/dL

Transferrin Saturation

19.75

16 - 55

Calculated

%

Vitamin B12, Cyanocobalamin

338

187 - 883

Serum, Chemiluminescent Microparticle Immunoassay (CMIA)

pg/mL

Vitamin D, 25-Hydroxy

21.3

Deficient: < 20
Insufficient: 20-30
Sufficient: 30-100
Potential Toxicity: > 100
ng/mL

Serum, Chemiluminescent Microparticle Immunoassay (CMIA)

Clinical Significance - Vitamin D, Total

For the diagnosis of vitamin D deficiency, it is recommended that there be a clinical correlation with serum 25(OH)vitamin D, calcium, parathyroid hormone, and alkaline phosphatase.

While monitoring oral vitamin D therapy, serum 25(OH)vitamin D should be tested after 3 months of treatment. However, the required dosage of vitamin D supplements and time to achieve target vitamin D levels show seasonal and individual variability depending on age, body fat, sun exposure, physical activity, genetic factors (especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes, and calcium or magnesium deficiency influencing the metabolism of vitamin D.

Ankit Prakash**25 Year(s)/Male**

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**Test****Result****Biological Reference Interval**

Approved By

Dr ShivaprakashMSc PhD (Medical Biochemistry)
Biochemist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 03:16 PM

Reported : 06/09/2026 08:27 PM



MC-6367



Test	Result	Biological Reference Interval
Homocysteine (HCY) Serum,Enzymatic: (CBS, CBL and LDH)	14.25	6.6 - 14.8 μmol/L

Approved By

Dr. Aekta
MBBS, MD (Pathology)
Pathologist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 02:48 PM

Reported : 06/09/2026 03:47 PM



Test	Result	Biological Reference Interval
Folic Acid Folate Vitamin B9 Serum, Chemiluminescent Immunoassay	<u>3.83</u>	3.96 - 16.76 ng/mL

Clinical Significance - Folic Acid | Folate | Vitamin B9

1. Folic acid is vital for normal function of red blood cells (RBCs) and white blood cells (WBCs) and for the adequate synthesis of certain purines and pyrimidines, which are precursors of deoxyribonucleic acid (DNA).
2. Folic acid is present in a wide variety of foods such as dark, leafy vegetables, citrus fruits, yeast, legumes, eggs, and milk. The predominant form of circulating folate is 5-methyltetrahydrofolic acid (5-mTHF). A methyl group is transferred from 5-mTHF to cobalamin in the pathway that links metabolism of folic acid and vitamin B12. Vitamin B12 is necessary for conversion of inactive 5-mTHF to active tetrahydrofolate, the active form of folate.
3. Decreased levels may be due to malnutrition, malabsorption syndrome (sprue, celiac disease), pregnancy, megaloblastic anaemia, hemolytic anaemia, malignancy, liver disease or chronic renal disease
4. Inadequate folate intake has been implicated in the development of neural tube defects or enhancement of certain types of cancer. Folate deficiency and low folate status have been linked in clinic studies to depression, persistent depressive symptoms, and poor antidepressant response.
5. Some drugs that may cause decreased folic acid levels include alcohol, aminopterin, amino-salicylic acid, ampicillin, antimalarial, chloramphenicol, erythromycin, estrogens, methotrexate, oral contraceptives, penicillin, phenobarbital, phenytoin, and tetracyclines.

Approved By

Dr. Deepak S
MBBS, MD (Pathology)
Pathologist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 07:43 PM

**Test****Result****Biological Reference Interval****Metals****Magnesium (Mg)**

Serum, Colorimetric - Forzman Dye

2.1**1.6 - 2.3**

mg/dL

Approved By

Dr Shivaprakash

MSc PhD (Medical Biochemistry)

Biochemist

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 08:25 PM



Test

Result

Biological Reference Interval

Cardiovascular Profile

High Sensitivity C-Reactive Protein
(HSCRP)

Serum, Immunoturbidimetry

1.259

Low: < 1

Average: 1 - 3

High: > 3

mg/L

Clinical Significance - High Sensitivity C-Reactive Protein (HSCRP)

hsCRP (mg/L)	Cardiovascular Risk
<1.0	Low
1-3	Average
3.1-10	High
>10	Persistent elevation may represent non-cardiovascular inflammation

High sensitive CRP (hsCRP) assay is used as an aid in the identification and stratification of individuals at risk for future cardiovascular disease.

Measurement should be done twice (averaging results) optimally two weeks apart, on metabolically stable patients, and should be compared to previous values.

Testing should not be performed while there is indication of infection, systemic inflammation, or trauma.

Lipid Profile

Serum

Cholesterol, Total

Cholesterol Esterase/Cholesterol Oxidase/Peroxidase

167

< 200

mg/dL

Triglycerides

Cholesterol Oxidase

105

< 150

mg/dL

High-Density Lipoprotein (HDL)
Cholesterol

Cholesterol Esterase/Cholesterol Oxidase/Peroxidase

48

> 50

mg/dL

Non-High Density Lipoprotein (Non-
HDL) Cholesterol

Calculated

119

< 130

mg/dL

Low-Density Lipoprotein (LDL)
Cholesterol

Calculated

97

< 100

mg/dL

Very Low-Density Lipoprotein (VLDL)
Cholesterol

Calculated

21

< 30

mg/dL

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Reported : 06/09/2026 08:25 PM



Test	Result	Biological Reference Interval
Cholesterol/High Density Lipoprotein (HDL) Ratio Calculated	<u>3</u>	3.3 - 4.4
Low-Density Lipoprotein/High-Density Lipoprotein (LDL/HDL) Ratio Calculated	2.0	0.5 - 3
High-Density Lipoprotein/Low-Density Lipoprotein (HDL/LDL) Ratio Calculated	0.5	> 0.4 Ratio

Clinical Significance - Lipid Profile

Remarks	Total Cholesterol (mg/dL)	Triglycerides (mg/dL)	LDL Cholesterol (mg/dL)
Optimal	<200	<150	<100
Above Optimal	-	-	100-129
Borderline	200-239	150-199	130-159
High	≥ 240	200-499	160-189
Very High	-	≥ 500	≥ 190

Lipid profile is a group test consisting of various lipids. Lipid profiles are generally collected with overnight fasting. However, recent guidelines have recommended non fasting samples for lipid profile for assessment of cardiovascular risk. The details for the study can be checked at

<https://jamanetwork.com/journals/jamainternalmedicine/fullarticle/2733560>

In certain instances measurements in the same patient can show physiological and analytical variations. In such cases three serial samples at an interval of 1 week each are recommended for Total cholesterol, TG, HDL and LDL.

Cholesterol levels are increased in primary hypercholesterolemia; secondary hyperlipoproteinemia, including nephrotic syndrome; primary biliary cirrhosis; hypothyroidism; and in some cases, diabetes mellitus. Low cholesterol levels may be found in malnutrition, malabsorption, advanced malignancy, and hyperthyroidism.

Triglyceride levels are used in the diagnosis and treatment of patients with diabetes mellitus, nephrosis, liver obstruction, other diseases involving lipid metabolism, or various endocrine disorders.

High Density Lipoprotein (HDL) cholesterol levels is used to evaluate the risk of developing coronary heart disease (CHD). The risk of CHD increases with lower HDL cholesterol concentrations.

LDL (low-density lipoprotein) cholesterol level, sometimes called "bad" cholesterol, makes up most of our body's cholesterol. High levels of LDL cholesterol raise your risk for heart disease and stroke.

Very-low-density lipoprotein (VLDL) cholesterol is produced in the liver and released into the bloodstream to supply body tissues with triglycerides. High levels of VLDL cholesterol have been associated with the development of plaque deposits on artery walls, which narrow the passage and restrict blood flow.

Apolipoprotein B/A1 (Apo B/A1) Ratio | Serum

Apolipoprotein B (Apo B)

Immunoturbidimetry

122.56

60 - 140

mg/dL

Ankit Prakash

25 Year(s)/Male

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Test	Result	Biological Reference Interval
Apolipoprotein A1 (Apo A1) Immunoturbidimetry	131.90	105 - 175 mg/dL
Apolipoprotein B/A1 (Apo B/A1) Ratio Calculated	0.93	0.35 - 0.98 Ratio
High Sensitive Troponin I Serum,Enhanced Chemiluminescence	<1.50	< 12 ng/L
N-Terminal Pro B-Type Natriuretic Peptide (NT-pro BNP) Serum,Enzyme-Linked Fluorescence Assay (ELFA)	30	pg/mL

Clinical Significance - N-Terminal Pro B-Type Natriuretic Peptide (NT-pro BNP)

Category		Source
Chronic heart failure	Optimal cut off (pg/mL)	European Society of Cardiology guidelines 2012.
Age <75 years	125 pg/mL	ESC recommends 125 pg/mL as single age independent cut off for exclusion of suspected heart failure in patients with non-acute onset.
Age ≥75 years	450 pg/mL	
Acute heart failure	Optimal cut off (pg/mL)	
Age <50 years	450 pg/mL	European Society of Cardiology guidelines 2012. ESC recommends 300 pg/mL as single age independent cut off for exclusion of suspected heart failure patients presenting with acute symptoms
Age 50-75 years	900 pg/mL	
Age >75 years	1800 pg/mL	

- NT-proBNP is a substance produced by left ventricle of the heart which helps to relax the blood vessels for maintaining smooth blood flow. It helps in detection, diagnosis and evaluation of congestive heart failure.
- NT-proBNP can also be elevated in pulmonary embolism, hypertension, valvular heart disease, heart muscle disease, arrhythmias, critical illness like sepsis, anaemia and stroke.
- Low levels of NT-proBNP can be seen in Obesity, pulmonary oedema, pericarditis and end stage cardiomyopathy.

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**Test****Result****Biological Reference Interval**

Approved By

Dr ShivaprakashMSc PhD (Medical Biochemistry)
Biochemist

Ankit Prakash

25 Year(s)/Male

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Reported : 06/09/2026 02:36 PM



Test

Result

Biological Reference Interval

Liver and Pancreatic Profile

Amylase

Serum, Twopoint-Rate-Dyed Amylopectin

74

30 - 110

U/L

Lipase

Serum, Twopoint-Rate with Colipase

212

23 - 300

U/L

Liver Function Test (LFT) | Serum

Bilirubin, Total

Diazo Method

0.83

0.2 - 1.3

mg/dL

Bilirubin, Direct

Calculated

0.41

0 - 0.3

mg/dL

Bilirubin, Indirect

Reflectance Spectrophotometry

0.42

0.1 - 1.1

mg/dL

Aspartate Aminotransferase (AST/SGOT)

Multipoint-Rate/UV with Pyridoxal-5-Phosphate (P-5-P)

37

17 - 49

U/L

Alanine Transaminase (ALT)

LDH, UV Kinetic

57

<50

U/L

Aspartate Aminotransferase/Alanine Transaminase (AST/ALT) Ratio

Calculated

0.6

0.7 - 1.4

Alkaline Phosphatase (ALP)

p-Nitrophenyl Phosphate

99

38 - 126

U/L

Gamma-Glutamyl Transferase (GGT)

SZAZ Carboxylated Substrate

18

15 - 73

U/L

Protein

Biuret

7.6

6 - 8.3

g/dL

Ankit Prakash

25 Year(s)/Male

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Test	Result	Biological Reference Interval
Albumin Bromo-Cresol Green	4.8	3.5 - 5 g/dL
Globulin Calculated	2.7	2.3 - 3.5 g/dL
Albumin/Globulin (A/G) Ratio Calculated	1.8	0.8 - 2 Ratio

Clinical Significance - Liver Function Test (LFT)

In certain individuals, total bilirubin up to 2.0 mg/dl is considered normal. High bilirubin values can be due to jaundice.

Total bilirubin is invariably increased in jaundice. Causes of jaundice are prehepatic, resulting from various hemolytic diseases; hepatic, resulting from hepatocellular injury or obstruction; and posthepatic, resulting from obstruction of the hepatic or common bile ducts.

Increased direct bilirubin levels can occur in hepatobiliary disorders, including intrahepatic and extrahepatic biliary tree obstruction, liver cell damage, Dubin-Johnson syndrome, and Rotor syndrome.

High indirect bilirubin levels can occur in hemolytic disorders, Gilbert's syndrome, Crigler-Najjar syndrome, neonatal jaundice, and ineffective erythropoiesis.

High Aspartate Aminotransferase values can occur in Myocardial infarction, pulmonary emboli, skeletal muscle trauma, alcoholic cirrhosis, viral hepatitis, or drug-induced hepatitis.

Elevated Alanine Aminotransferase levels are seen in liver cell necrosis, hepatitis, hepatic cirrhosis, liver tumours, obstructive jaundice, Reye's syndrome, extensive trauma to skeletal muscle, myositis, myocarditis, or myocardial infarction.

High alkaline phosphatase levels can be due to primary and secondary hyperparathyroidism, Paget's disease of bone, carcinoma metastatic to the bone, osteogenic sarcoma, Hodgkin's disease, Hepatobiliary diseases involving cholestasis, inflammation, or cirrhosis.

ALP levels can also be elevated in fever and increased bone metabolism (e.g., in adolescents and during the healing of a fracture), in renal infarction and failure and in pregnancy complications.

Low ALP levels may occasionally be seen in hypothyroidism.

Gamma-glutamyl transferase (GGT) is a sensitive indicator of hepatobiliary disease. It is useful in the diagnosis of obstructive jaundice and chronic alcoholic liver disease, in the follow-up of chronic alcoholics undergoing treatment, and in the detection of hepatotoxicity. GGT is more responsive to biliary obstruction than AST, ALT, or ALP.

Total protein levels can be used to evaluate nutritional status.

High protein concentrations can be due to dehydration, Waldenström's macroglobulinemia, multiple myeloma, hyperglobulinemia, granulomatous, and some tropical diseases.

Low protein concentrations can be due to pregnancy, excessive intravenous fluid administration, cirrhosis or other liver diseases, chronic alcoholism, heart failure, nephrotic syndrome, glomerulonephritis, neoplasia, protein-losing enteropathies, malabsorption, and severe malnutrition.

Increased albumin levels may indicate dehydration or hyperinfusion with albumin.

Decreased albumin levels are found in rapid or over-hydration, severe malnutrition and malabsorption, severe diffuse liver necrosis, chronic active hepatitis, and neoplasia.

Albumin is commonly reduced in chronic alcoholism, pregnancy, renal protein loss, thyroid dysfunction, peptic ulcer disease, and chronic inflammatory diseases.

Globulin includes carrier proteins, enzymes, complement, and immunoglobulins. Most of these are synthesised in the liver, although immunoglobulins are synthesised by plasma cells.

Increased globulin level usually results from an increase in immunoglobulins.

Malnutrition and congenital immune deficiency can decrease globulin levels due to decreased synthesis. Nephrotic syndrome can cause decreased globulin levels due to protein loss through the kidney.

AST/ALT Ratio > 2:1 (AST is two times higher than ALT) is indicative of alcoholic liver disease.

AST/ALT Ratio < 1:1 (ALT is higher than AST) indicates non-alcoholic fatty liver disease.

Ankit Prakash**25 Year(s)/Male**

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**Test****Result****Biological Reference Interval**

Approved By

Dr ShivaprakashMSc PhD (Medical Biochemistry)
Biochemist

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

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Patient ID : OHP0BG991415136

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Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 02:59 PM



Test

Result

Biological Reference Interval

Diabetic Profile

Glycated Hemoglobin (HbA1C) | Whole Blood EDTA

Glycated Hemoglobin (HbA1C)

High-Performance Liquid Chromatography (HPLC)

5.5

Normal: < 5.7

Pre-Diabetes: 5.7-6.4

Diabetes: $\Rightarrow$ 6.5

%

Mean Blood Glucose

Calculated

111

< 117

mg/dL

Homeostasis Model Assessment - Insulin Resistance (HOMA-IR) | Fluoride Plasma

Glucose, Fasting

Glucose Oxidase-Peroxidase (GOD-POD)

82

70 - 99

mg/dL

Insulin, Fasting

Chemiluminescent Microparticle Immunoassay (CMIA)

12.5

2-25

 μ U/mL

Beta Cell Function

Calculated

159.1

%

Insulin Sensitivity

Calculated

63.7

%

Homeostasis Model Assessment -
Insulin Resistance (HOMA-IR)

Calculated

1.5

0.7 - 2

Approved By

Dr. Rachana
MBBS, MD (Pathology)
Pathologist

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

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Patient ID : OHP0BG991415136

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Reported : 06/09/2026 02:37 PM



Test

Result

Biological Reference Interval

Kidney Profile

Kidney Function Test with Electrolytes (KFT / RFT) | Serum

Urea Urease	39	19 - 43 mg/dL
Creatinine Twopoint-Rate-Creatinine Aminohydrolase	1.0	0.66 - 1.25 mg/dL
Blood Urea Nitrogen (BUN) Calculated	18.2	6 - 20 mg/dL
Blood Urea Nitrogen (BUN)/Creatinine Ratio Calculated	18.20	10 - 20 Ratio
Uric Acid Uricase	6.5	3.5 - 8.5 mg/dL
Calcium Arsenazo Method	9.2	8.4 - 10.2 mg/dL
Phosphorus Phosphomolybdate Formation	3.4	2.5 - 4.5 mg/dL
Estimated Glomerular Filtration Rate (eGFR) Twopoint-Rate-Creatinine Aminohydrolase/Calculation	107	Normal: $\Rightarrow$ 90 Mild decrease: 60-89 Mild moderate decrease: 45-59 Severe decrease: 15-29 End stage kidney disease: < 15 ml/min/1.73m ²

Electrolytes

Sodium Direct ISE	142	137 - 145 mmol/L
Potassium Direct ISE	4.6	3.5 - 5.5 mmol/L

Ankit Prakash

25 Year(s)/Male

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Collected : 06/09/2026 10:20 AM

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Reported : 06/09/2026 02:37 PM



Test	Result	Biological Reference Interval
Chloride Direct ISE	105	98 - 107 mmol/L

Microalbumin Creatinine Ratio | Urine

Microalbumin Immunoturbidimetry	0.6	< 30 mg/dL
Creatinine - Urine Twopoint-Rate-Creatinine Aminohydrolase	36.6	24 - 292 mg/dL
Microalbumin/Creatinine Ratio Calculated	16.4	Microalbuminuria: 30-300 Macroalbuminuria: > 300 Normal: < 30 mg/g Creatinine

Urine Complete Analysis | Urine

Urine, Physical Examination

Colour Manual	Pale yellow	Pale yellow
Appearance RGB Sensor Technology	Clear	Clear

Urine, Chemical Examination

pH pH Indicatory Paper	6.5	5 - 8
Specific Gravity Bromo Thymol Blue Indicato	1.005	1.001 - 1.035
Protein Protein Error of pH Indicator	Nil	Nil

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 02:37 PM



Test	Result	Biological Reference Interval
Glucose Enzyme Method Glucose Oxidase-Peroxidase (GOD-POD)	Nil	Nil
Ketones Nitroprusside Method/Dipstick	Nil	Nil
Bilirubin Azo Coupling Method	Nil	Nil
Blood Peroxidase Activity	Negative	Negative
Urobilinogen Azo Coupling Method	Normal	Normal
Leucocyte Esterase Granulocyte Esterase Method	Negative	Negative
Nitrites Griess Method	Negative	Negative
Urine, Microscopic Examination		
Pus Cells Automated Morphological Microscopy	0-1	0-5 /hpf
Epithelial Cells Automated Morphological Microscopy	0-1	0-2 /hpf
Red Blood Cells (RBC) Automated Morphological Microscopy	0-1	0-2 /hpf
Granular Casts Automated Morphological Microscopy	Nil	Nil /hpf
Hyaline Casts Automated Morphological Microscopy	Nil	Occasional /hpf
Uric Acid Crystals Automated Morphological Microscopy	Nil	Nil /hpf

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

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Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

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Reported : 06/09/2026 02:37 PM



Test	Result	Biological Reference Interval
Phosphate Crystals Automated Morphological Microscopy	Nil	Nil /hpf
Calcium Oxalate Crystals Automated Morphological Microscopy	Nil	Nil /hpf
Amorphous Urates Automated Morphological Microscopy	Nil	Nil /hpf
Amorphous Phosphates Automated Morphological Microscopy	Nil	Negative /hpf
Yeast Automated Morphological Microscopy	Nil	Nil /hpf
Bacteria Automated Morphological Microscopy	Nil	Nil /hpf
Parasites Automated Morphological Microscopy	Nil	Nil /hpf
Mucus Manual	Absent	Absent

Approved By

Dr Shivaprakash

MSc PhD (Medical Biochemistry)

Biochemist

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 04:40 PM



Test

Result

Biological Reference Interval

Thyroid Profile

Thyroid Profile, Free | Serum

Triiodothyronine (T3), Free

2.47

1.58 - 3.91

Chemiluminescent Microparticle Immunoassay (CMIA)

pg/mL

Clinical Significance - Triiodothyronine (T3), Free

In pregnant females the reference range of Free T3 differs. Please refer the table below for the same:-

PREGNANCY	REFERENCE RANGE (pg/mL)
1st Trimester	2.88-4.32
2nd Trimester	2.56-4.41
3rd Trimester	2.47-4.73

Thyroxine (T4), Free

1.13

0.7 - 1.48

Chemiluminescent Microparticle Immunoassay (CMIA)

ng/dL

Clinical Significance - Thyroxine (T4), Free

In pregnant females the reference range of Free T4 differs. Please refer the table below for the same:-

PREGNANCY	REFERENCE RANGE (ng/dL)
1st Trimester	0.85-1.64
2nd Trimester	0.78-1.48
3rd Trimester	0.68-1.41

Thyroid Stimulating Hormone (TSH)

3.1248

0.35 - 4.94

Chemiluminescent Microparticle Immunoassay (CMIA)

μIU/mL

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

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Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 04:40 PM



Test

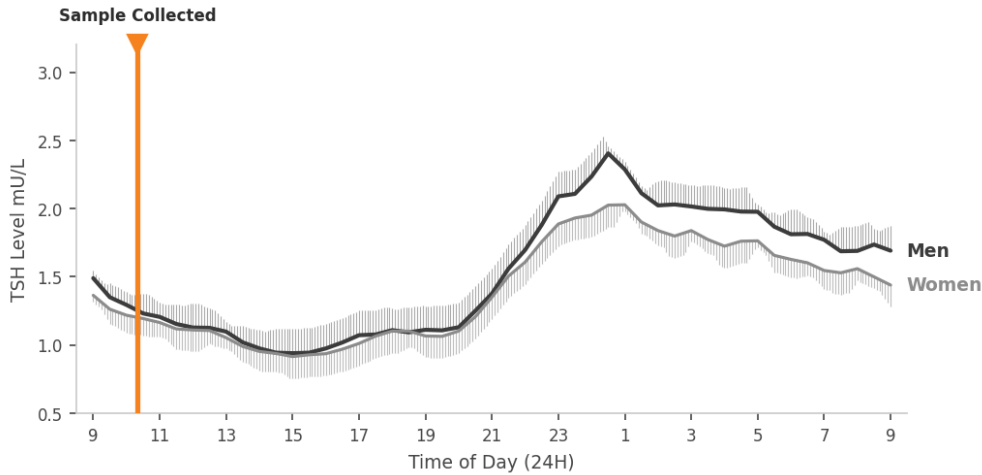
Result

Biological Reference Interval

Clinical Significance - Thyroid Profile, Free

EXCLUSIVE

1. Performed using a highly sensitive third-generation TSH assay.



1. High TSH usually indicates an underactive thyroid (primary hypothyroidism).
2. Low TSH usually indicates an overactive thyroid (hyperthyroidism).

Factors influencing TSH values:

1. Food intake
2. Circadian rhythm (up to 70%)
3. Medications
4. Physiological & clinical conditions like stress, pregnancy, iodine imbalance, genetic factors, etc.

In pregnant females, the reference range of TSH differs. Please refer to the table below:

PREGNANCY	TSH REFERENCE RANGE (μIU/mL)
1st Trimester	0.100–2.500
2nd Trimester	0.200–3.000
3rd Trimester	0.300–3.000

References:

1. Indian Journal of Endocrinology and Metabolism 18(5): p. 705–707, Sep–Oct 2014.
2. http://www.pnei-it.com/1/upload/thyrotropin_secretion_patterns_in_health_and_disease.pdf

Triiodothyronine (T3), Total

0.94

0.35 - 1.93

Serum, Chemiluminescent Microparticle Immunoassay (CMIA)

ng/mL

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

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Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

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Reported : 06/09/2026 04:40 PM

**Test****Result****Biological Reference Interval****Clinical Significance - Triiodothyronine (T3) - Total**

First Trimester:	0.8 - 1.9 ng/mL
Second Trimester:	1.0 - 2.6 ng/mL
Third trimester:	1.0 - 2.6 ng/mL

Thyroxine (T4), Total**6.77****4.87 - 11.72**

Serum, Chemiluminescent Microparticle Immunoassay (CMIA)

µg/dL

Clinical Significance - Thyroxine (T4) - Total

PREGNANCY	TSH REFERENCE RANGE(µIU/ml)
1st Trimester	0.100-2.500
2nd Trimester	0.200-3.00
3rd Trimester	0.300-3.00

Note:

1. TSH has a circadian rhythm, with a peak around midnight (with much variability between individuals), and low in the afternoon. Secretory pulses occur every 2-3hrs and are interspersed with periods of tonic non-pulsatile TSH secretions. The change in TSH from peak to trough is approximately 72%.
2. TSH may be transiently altered because of non thyroidal illness like infections, recovery phase of illness, surgery.

Thyroid Peroxidase Antibody (TPO)**4.7****< 8**

Serum, Enzyme-Linked Fluorescence Assay (ELFA)

IU/mL

Approved By

Dr Shivaprakash

MSc PhD (Medical Biochemistry)

Biochemist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 01:22 PM



Test

Result

Biological Reference Interval

Hormonal Profile**Testosterone****469.84****260-1000**

Serum, Chemiluminescent Microparticle Immunoassay (CMIA)

ng/dL

Approved By

*Rachana L.Y.***Dr. Rachana**

MBBS, MD (Pathology)

Pathologist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 02:48 PM

Reported : 06/09/2026 07:43 PM



Test	Result	Biological Reference Interval
Cortisol, Random Serum,Chemiluminescent Immunoassay	13.10	Morning sample: 4.5-24 Evening sample: 1.8-6.5 µg/dL
Testosterone, Free Serum,Enzyme-Linked Immunosorbent Assay (ELISA)	8.48	5.01 - 27.78 pg/mL
Dehydroepiandrosterone Sulphate (DHEAS) Serum,Chemiluminescent Immunoassay	182.9	Refer below µg/dL

Clinical Significance - Dehydroepiandrosterone Sulphate (DHEAS)

Reference range:

Age in years	Males(microgram/dL) for serum samples	Females(microgram/dL) For serum samples
3-9	1.7 - 61.8	2.1 - 79
9-18	21 - 397.7	16 - 189.6
20-29	161.2 - 561.6	80.2 - 339.5
30-39	124.5 - 482.7	58.7 - 227
40-49	96.9 - 391.5	46.7 - 247.6
50-59	59.7 - 307.9	33 - 212.8
>60	40.8 - 405.4	61.6 - 124.1

Interpretation:

Dehydroepiandrosterone (DHEA) is a steroid prohormone synthesised mainly by the adrenal cortex starting from cholesterol sulphate, pregnenolone sulphate and 17-hydroxypregnenolone sulphate. DHEA is a lipophilic molecule that can be converted into its circulating hydrophilic form, dehydroepiandrosterone sulphate (DHEA-S). Conversion into the hydrophilic storage form occurs in adrenals, liver and small intestine.

Although HAMA-neutralising agents are added, extremely high HAMA (human anti-mouse antibodies) concentrations may occasionally influence results. The long half life of serum DHEA-S and the limited diurnal variations make DHEA-S a good marker for the assessment of adrenal cortical function.

The main indications for DHEA-S assay are for the diagnosis of endocrine pathologies, especially those of adrenal origin:

- Evaluation of puberty. DHEA-S levels are low in case of delayed puberty and high in case of precocious puberty.
- Evaluation of adrenal function: DHEA-S levels are low in primary and secondary hypoadrenalism; conversely, DHEA-S is the only androgen present in abnormally high concentrations in hirsute amenorrhoeic women in whom the syndrome is of adrenal origin, and in patients affected by congenital adrenal hyperplasia (CAH).
- Most patients affected by Cushing's disease (pituitary aetiology) have moderately elevated levels of DHEA-S. High levels of DHEA-S are often found in polycystic ovary syndrome. Very high levels of DHEA-S in women are suggestive of a hormone-secreting adrenal tumour.
- Virilizing tumours of the ovary are associated with marked increases in serum testosterone, but near normal DHEA-S levels.

Sex Hormone Binding Globulin (SHBG)

Serum,Chemiluminescent Immunoassay

28.4**14.4-70.7**

nmol/L

Ankit Prakash

25 Year(s)/Male

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 02:48 PM

Reported : 06/09/2026 07:43 PM



Test

Result

Biological Reference Interval

Approved By

Dr Drisya
MBBS, MD (Pathology)
Pathologist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 12:19 PM

Reported : 06/09/2026 02:54 PM



Test

Result

Biological Reference Interval

Immune Profile**Rheumatoid Factor (RF)**

Serum, Immunospectrophotometry

<8.6

< 14

IU/mL

Approved By

Dr ShachiMBBS, MD (Pathology)
Pathologist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

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Reported : 06/09/2026 01:22 PM

**Test****Result****Biological Reference Interval****Tumor Markers****Prostate Specific Antigen (PSA)**

Serum, Chemiluminescent Microparticle Immunoassay (CMIA)

0.439**<4**

ng/mL

Approved By

*Rachana L.Y.***Dr. Rachana**
MBBS, MD (Pathology)
Pathologist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 03:16 PM

Reported : 06/09/2026 07:57 PM



Test	Result	Biological Reference Interval
Carcinoembryonic Antigen (CEA) Serum,Chemiluminescent Immunoassay	1.58	<4 ng/mL
Carbohydrate Antigen 19-9 (CA 19-9) Serum,Chemiluminescent Immunoassay	17.0	0 - 37 U/mL
Alpha Feto Protein (AFP) Serum,Chemiluminescent Immunoassay	1.30	<5.5 IU/mL

Clinical Significance - Alpha Feto Protein (AFP)

AFP in maternal Serum(IU/mL)	
14	22.5
15	23.1
16	24.9
17	28.9
18	33.9
19	35.9
20	42.3
21	53.3
22	64.5
23	60.5

- AFP, which belongs to the group of oncofetal proteins, is produced by the yolk sac and in the foetal liver. Useful for determining prognosis and monitoring therapy for Hepatocellular carcinoma. Level of AFP is a prognostic indicator of survival. Elevated AFP and serum bilirubin levels in these patients is associated with shorter survival time. Measurement of AFP levels in combination with HCG levels are useful in classifying and staging Germ cell tumours. To predict tumour recurrence/presence of residual tumors. Increased in Germ cell (Non-Seminomatous) tumours, Primary hepato cellular carcinoma (70%), Teratocarcinoma, Gastrointestinal tract cancers with or without liver metastasis, Benign hepatic conditions like Acute Viral Hepatitis, Chronic active hepatitis and Cirrhosis. During pregnancy, AFP levels in maternal blood continuously increase. Between weeks 28 to 32, a maximum is reached: after this period, a decrease can be observed until delivery. In the amniotic fluid, the maximum is achieved between the 13th and 15th week of gestation.
- Elevated AFP levels in early pregnancy indicate neural tube defects (spina bifida, anencephaly). Lower AFP concentrations in maternal serum are indicative of Down's syndrome. The determination of serum AFP during therapeutic monitoring provides valuable information about the success or failure of treatment as well as about recurrence.
- This test is not used for screening.
- Follow up and comparison has to be done on similar platform/equipment.

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 03:16 PM

Reported : 06/09/2026 07:57 PM

**Test****Result****Biological Reference Interval**

Approved By

Dr. Aekta
MBBS, MD (Pathology)
Pathologist

Ankit Prakash**25 Year(s)/Male**

Ref. by :

Partner : -

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 06/09/2026 10:20 AM

Received : 06/09/2026 03:16 PM

Reported : 06/09/2026 07:57 PM



Test

Result

Biological Reference Interval

Reviewed By

Dr. Sayantani Sarkar
MBBS, MD (Pathology)
Pathologist**Dr Sindhu**
MBBS, MD (Pathology)
Pathologist**Dr. Aekta**
MBBS, MD (Pathology)
Pathologist**Dr. Rachana**
MBBS, MD (Pathology)
Pathologist**Dr. Alekya Seerapu**
MBBS, MD (Pathology)
Pathologist**Dr Shivaprakash**
MSc PhD (Medical Biochemistry)
Biochemist**Dr. Deepak S**
MBBS, MD (Pathology)
Pathologist**CONDITIONS OF LABORATORY TESTING & REPORTING**

- It is presumed that the test sample belongs to the patient named or identified in the test requisition form. Test results released pertain to the specimen submitted.
- Laboratory investigations are only a tool to facilitate arriving at a diagnosis and should be clinically correlated by the Referring Physician.
- All tests are performed and reported as per the turnaround time stated in the Orange Health Labs Directory of Services (DOS).
- Orange Health Labs confirms that all tests have been performed or assayed with the highest quality standards, clinical safety & technical integrity.
- All test results are dependent on the quality of the sample received by the Laboratory and the assay technology.
- Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.
- A requested test might not be performed if:
 - The specimen received is insufficient or inappropriate, or the specimen quality is unsatisfactory
 - Incorrect specimen type
 - Request for testing is withdrawn by the ordering doctor or patient
 - There is a discrepancy between the label on the specimen container and the name on the test requisition form
- Test results may show interlaboratory variations.
- Test results are not valid for medico-legal purposes.
- This is a computer-generated medical diagnostic report that has been validated by an Authorized Medical Practitioner/Doctor. The report does not need a physical signature.

REPORT PRINTOUT/HARDCOPY

- Patients can collect free report printouts from our Lab or the nearest Collection Centre.
- Hard copies can be delivered within 24 hours for ₹200/report (non-refundable after dispatch).
- For assistance, contact Orange Support at +91-9008111144.

Ankit Prakash

25 Year(s) / male

Srf ID:

Ref. by:

Partner :

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 2026-09-06 | 10:20 AM

Received : 2026-09-06 | 02:48 PM

Reported : 2026-09-07 | 11:56 AM



Investigation

Lipoprotein(a)

(Serum, Immunoturbidimetry)

Interpretation :

1. High levels are found in people at risk for stroke, heart attack or Coronary heart disease (CHD). Levels are also raised in renal failure, uncontrolled diabetes, hypercholesterolemia, hypothyroidism, estrogen depletion and nephrotic syndrome.

Clinical Utility:

1. Helps in early detection of potential cardiovascular problems.

2. Helps in monitoring management of individuals with a family history of Coronary heart disease.

Note: Serum concentration of Lp(a) are related to genetic factors and are largely unaffected by diet, exercise and lipid lowering drugs.

Disclaimer: Measuring Lp(a) may be reasonable in patients with a personal history of or first-degree relative with premature atherosclerotic cardiovascular disease and in severe hypercholesterolemia.

Associated Tests: Cholesterol (C0106), Lipid profile (L0071), APOB/A1 ratio (A0468), Homocysteine(H0254), HsCRP -High Sensitivity CRP Serum (H0265).

References:

1. Kit Insert

2. Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine. 11th ed. Philadelphia, PA: Elsevier; 2019:chap 48

-- End of Report --

Dr. RAVIKUMAR H N
MBBS, MD, DCP Pathology
Sr Consultant Pathologist
Reg No. KMC 22886

Ankit Prakash

25 Year(s) / male

Srf ID:

Ref. by:

Partner :

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 2026-09-06 | 10:20 AM

Received : 2026-09-06 | 02:46 PM

Reported : 2026-09-09 | 03:27 PM



Investigation

Zinc by ICPMS, Serum

(Serum, Inductively Coupled Plasma Mass Spectrometry (ICPMS))

Observed Value

96.64

Unit

µg/dL

Biological Reference Interval

54-151

Zinc Serum Interpretation	Associated conditions
Zinc is vital trace element required for normal healing of wounds and normal immune function. Zinc levels lowered with systemic infections & inflammatory disorders, oral contraceptives, and pregnancy.	Zinc deficiency - Symptoms include depressed growth, teratogenesis, poor carbohydrate metabolism, altered Cognition, poor immune function, alopecia, impotence, eye and skin lesions, and diarrhoea.
Zinc exposure can occur from Occupational exposure related to mining and metallurgic industries. Use of commercial products containing zinc (e.g. Zinc containing shampoos, Multivitamins)	Zinc excess is not of major clinical concern, however elevated zinc concentrations may interfere with copper absorption.

1. Whole Blood / Serum metal testing is used for the detection of recent exposure or poisoning with the toxic element. However, blood metal levels in healthy subjects can vary considerably with exposure to the particular metal present in the diet and in the environment.
2. It should be noted that low or within acceptable levels in blood / Serum do not always exclude that the element is uninvolved in contributing to the patient's symptoms because certain elements may be sequestered in tissues.
3. Lower metal levels in patients on follow-up imply that the toxic element exposure is reduced in the patient's immediate environment or that the body has efficiently eliminated the toxic element.

Reference -

1. Sample collection guidelines for trace elements in blood and urine. International union of pure and applied chemistry clinical chemistry division commission on toxicology working party. Pure & Appl. Chem., Vol. 67, Nos 8/9, pp. 1575-1608, 1995.
2. Nutrient & toxic elements interpretative guide, metamatrix, USA, 2011.

Mercury, Serum

(Serum, Inductively Coupled Plasma Mass Spectrometry (ICPMS))

0.7

µg/L

0.21-1.3

Mercury Serum Interpretation	Associated conditions
Mercury exposure can occur from Dental amalgams, Broken thermometers, barometers, contaminated sea food consumption, preservatives (esp. thimerosal), Grain seeds treated with methyl mercury fungicide.	Mercury toxicity is often manifested as Mental symptoms (insomnia, fatigue, poor short-term memory), tremor, stomatitis, gingivitis, GI and Renal disturbances and decreased immunity.

1. Whole Blood / Serum metal testing is used for the detection of recent exposure or poisoning with the toxic element. However, blood metal levels in healthy subjects can vary considerably with exposure to the particular metal present in the diet and in the environment.
2. It should be noted that low or within acceptable levels in blood / Serum do not always exclude that the element is uninvolved in contributing to the patient's symptoms because certain elements may be sequestered in tissues.
3. Lower metal levels in patients on follow-up imply that the toxic element exposure is reduced in the patient's immediate environment or that the body has efficiently eliminated the toxic element.

Dr. VARSHA BIRLA
MD, DNB (BIOCHEMISTRY)
HOD Clinical Chemistry
Reg no. 2010041116

Ankit Prakash

25 Year(s) / male

Srf ID:

Ref. by:

Partner :

Report Ref. ID : BLR6972960

Patient ID : OHP0BG991415136

Collected : 2026-09-06 | 10:20 AM

Received : 2026-09-06 | 02:46 PM

Reported : 2026-09-09 | 03:27 PM



Investigation

Observed Value

Unit

Biological Reference Interval

Reference –

1. Sample collection guidelines for trace elements in blood and urine. International union of pure and applied chemistry clinical chemistry division commission on toxicology working party. Pure & Appl. Chem., Vol. 67, Nos 8/9, pp. 1575-1608, 1995.
2. Nutrient & toxic elements interpretative guide, metamatrix, USA, 2011.

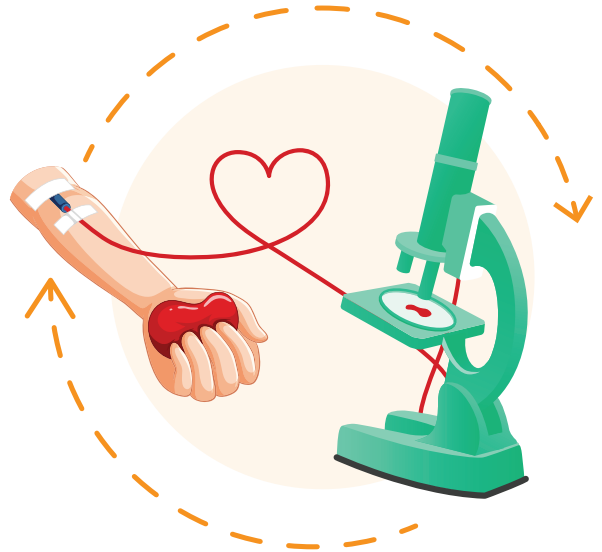
-- End of Report --

Dr. VARSHA BIRLA
MD, DNB (BIOCHEMISTRY)
HOD Clinical Chemistry
Reg no. 2010041116

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Blood starts deteriorating the minute it leaves the human vein, unless stored right. We have built our logistics to achieve the **fastest vein-to-machine testing time** in the industry, and in our journey ahead will strive to reduce it further as much as possible.



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We don't batch your samples to save costs. They are tested the moment they come in.

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Since we test your blood the fastest, you get the most accurate reports in the industry.